



PROCEDURE

Did Not Attend (DNA) Community Appointment

Scope (Staff):	NMHS Mental Health (MH) Staff: Community
Scope (Area):	NMHS Community Adult MH, Youth MH and Older Adult MH Services

1. Aim

All consumers who fail to attend (DNA) a scheduled appointment will have a review of current risk and based on the outcome of the risk evaluation, may need to be assertively followed up within 24 hours.

This procedure outlines how clinical staff should manage DNA's, including the development and implementation of an action plan that addresses any identified risks directly associated with the DNA.

This procedure is not applicable to scheduled appointments which are cancelled by consumers, please refer to [MH Consumer Declining Assessment of Follow Up Procedure](#) for further guidance.

2. Risk

One possible reason for a DNA, is that a consumer did not attend their scheduled appointment due to deterioration in their mental health. This procedure aims to reduce the risk of the consumer not receiving prompt follow up care. This procedure should be read in conjunction with the [MH Clinical Risk Assessment and Management Policy](#).

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Definitions

Review of risk: Identifying specific risk factors of relevance to an individual, and the context in which they may occur. This process requires linking historical information to current circumstances, to anticipate possible future risk.

Clinician: A health professional such as a medical practitioner, nurse, occupational therapist, psychologist, or social worker.

Community mental health service: A service that conducts assessment and provides treatment in the community for consumers whose primary concern is their mental health.

Consumer: A person who is currently accessing a community mental health service.

Multidisciplinary team (MDT): A group of health professionals who are members of different disciplines (psychiatrist, nurse, social worker etc.) each providing specific services to the consumer.

4. Process

4.1 When a Consumer DNA's

If a consumer does not attend (DNA) a scheduled appointment (any booked appointment, medical, psychology, case manager etc) the clinician who the consumer was booked to see will:

- Attempt direct contact with the consumer to ascertain the reason for the DNA.
- If the clinician is not able to establish contact with the consumer the clinician will use their current knowledge of the consumer and review the relevant clinical records as necessary to support their decision making.
- Considerations should be given to reviewing the following:
 - The historical and current risks (noted on the medical record, Psychiatric Services Online Information System PSOLIS and WebPSOLIS).
 - Treatment Support and Discharge Plan (TSDP)/Crisis Management Plan, with any specific reference to risks for the consumer in the event of a DNA.
 - Risk Assessment and Management Plan (RAMP) and Progressive Risk Assessments (PRA).
 - Recent assessments including direct conversations with other involved clinicians.
 - Recent progress notes (medical file).
 - Recent PSOLIS and WebPSOLIS entries.
 - Collateral information from referrers and stakeholders to assist with identifying the level and nature of current risks.

This review should be documented in the integrated progress notes in the medical record and on PSOLIS.

4.2 Risks identified

If risks are identified the clinician will:

- Assess the level of follow-up required for the consumer, as a matter of priority based on the above information that was gathered.

- If the consumer is unable to be contacted by telephone the clinician and or a member of the treating team may consider a home visit to conduct a welfare check or contact the family/carer, nominated person, personal support person or guardian, if appropriate.
- Discuss action plan with the consumer's treating medical practitioner and at the earliest scheduled acuity meeting (or clinical MDT meeting) with consideration given to:
 - requesting a Police Welfare check;
 - afterhours attempts to contact; and
 - review of WebPAS for hospital admissions.

4.3 No risk identified

If no risks are identified, the clinician/case manager will:

- Continue to attempt phone contact.
- Note the DNA at the weekly clinical meeting and discuss appropriate next steps as an MDT.
- Following several DNA's consideration should be given to sending a 2-week letter (or 1 week letter if deemed more appropriate by the team) following wider clinical discussion with the MDT (**see Appendix 1 & 2**).

All actions will be documented in the consumer's integrated progress notes.

5. Compliance, monitoring and evaluation.

Each Community MH Clinic will annually report via the Community MH Leadership Committee meetings:

- Monitor DNA occurrence and develop action plans as required to reduce DNAs.
- Review any clinical incidents relating to DNA's.

Related policies, procedures and guidelines

[Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy \(MP 0099/18\)](#)

[Safety Planning for Mental Health Consumers Policy \(MP0181/24\)](#)

[MH Clinical Risk Assessment and Management \(CRAM\) Policy](#)

[MH Case Management Procedure](#)

[MH Communication with General Practitioners \(GP\) and Shared Care Policy](#)

Useful resources

[Chief Psychiatrist of Western Australia – Standards For Clinical Care](#)

Integrated Progress Notes – PMR07

Progressive Risk Assessment (PRA)

Risk Assessment and Management Plan (RAMP) SMHMR05

Policy Sponsor	Co-Director, Community Adult MH				
Policy Contact	Service Manager, Community Adult MH				
Date First Issued:	09/10/2013	Last Reviewed:	31/05/2024	Review Date:	27/05/2027
Approved by:	MH Policy Coordination Committee			Date:	20/03/2024
Endorsed by:	MHPHDS Executive Director			Date:	31/05/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety			☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration	
Chief Psychiatrist's Standards for Clinical Care:	Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management,				
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Document Version Control			
Date	Version	Author	Notes
07/06/2018	1.2	A/Policy Officer	Review date amended to reflect endorsement date. MHA hyperlink repaired.
19/02/2019	2.0	Program Manager, Joondalup CMHS, NMHS MH	Scheduled review: Updated template, hyperlinks and NSQHS V2 standards, addition of content to improve clarity of process, change of policy type to procedure.
Feb 2021	2.1	Program Manager, Community Adult MH	Contemporised document.
Feb 2021	2.2	Program Manager, Community Adult MH	Stakeholder review by Program Managers Subspecialties, Youth, Forensic, Older Adult and Community Adult
16/03/2021	2.3	Program Manager, Community Adult MH	Updated links.

17/03/2021	2.4	A/Policy & Audit Officer	Transferred draft to new template for submission. Routine review by MH PCC
19/10/2022	3.0	Service Coordinator Community Mental Health Services	Scheduled review, update of DNA reasons, processes updated with inclusion of RAMP (previously BRA), PSOLIS Progressive Risk Assessment (PRA) added as per the updated CRAM Policy, inclusion of DNA letter templates and hyperlinks updated
31/05/2024	4.0	Service Manager Community Mental Health Services	Scheduled review, update of definitions, processes and compliance, inclusion of Older Adult MHS and Youth MHS..

This document can be made available in alternative formats on request for a person with a disability.

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Appendix 1: Letter Template Requesting Contact (2 week letter)



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services



Dear

We recently received a referral from ***** referring you to
***** Community Mental Health Clinic.

I have attempted to contact you via phone***** to get more information
to proceed with the referral, but have been unsuccessful in making contact with
you.

If you wish to engage in an assessment we encourage you to make contact at
a convenient time for you so we can identify your current needs. If you wish to
proceed with the referral, please call the Triage Officer on *****.

If you do not respond by ***** we will assume you do not wish to
engage with the clinic and we will therefore close the referral.

If you need to access services regarding any mental health concerns, please
see your GP or contact the Mental Health Emergency Response Line
(MHERL) on 1300 555 788 (brochure enclosed). Someone is available at
MHERL 24 hrs a day 7 days a week to take your call and provide you with
support and guidance. Or in the case of a mental health emergency you can
self-present at your local Emergency Department.

Yours sincerely,

Name
Triage Officer
***** Community Mental Health Clinic



Appendix 2: Letter Template (DNA) Appointment



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services



Dear

We wanted to make formal contact with you, as you (did not/ have not) attended the following appointment (s) *****

Based upon this, if you are wanting to remain engaged with ***** Community Mental Health Service could you please contact the clinic within the next two weeks on ***** , otherwise we will assume that you no longer wish to engage with the service.

If we do not hear from you before ***** you will be discharged back to your nominated GP for mental health follow up.

Please find enclosed MHERL leaflet for your reference in the interim. You can contact the Mental Health Emergency Response Line on 1300 555 788, someone is available 24 hrs a day 7 days a week to take your call and provide you with support and guidance. Or in the case of a mental health emergency you can self-present at your local Emergency Department.

Yours sincerely,

Name *****

Role *****

***** Community Mental Health Clinic

Team *****

